

MEDCORE HOSPITALS

Patient Communication Guidelines

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Effective Date	01 September 2026	Department	Patient Services
Classification	Internal	Owner	Patient Services

Document Control

Controlled internal guideline for patient-facing communication. Staff should use the current version and approved communication channels.

1. Purpose and Scope

These guidelines establish a consistent standard for patient-facing communication. They apply to reception, Patient Services, scheduling, billing-facing staff and other authorized employees. Communication should be clear, factual, respectful, appropriately limited and aligned with the employee's role.

Control	Standard practice	Responsible role	Escalation trigger
Routine	Follow the approved workflow and document material actions.	Process owner	Exception cannot be resolved.
Verification	Use authorized information and do not guess.	Assigned staff	Identity or source is uncertain.
Traceability	Keep status, owner and next step visible.	Department manager	Case becomes overdue or conflicting.

2. In-Person Communication

Staff should identify the request, determine whether verification is required and use an appropriate setting for sensitive discussion. Patients should receive a clear next step. Staff should not argue, speculate or promise outcomes outside their authority.

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3. Telephone Verification

Before discussing sensitive information by telephone, staff should follow the approved verification process. If verification fails, only general information should be provided. Requests requiring clinical interpretation should be routed to the Clinical Team.

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4. Written Communication

Approved templates and channels should be used. Recipient details should be checked and only necessary information should be included. Material messages, commitments and escalations should be documented where required.

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5. Digital Communication

Digital messages should follow the same verification, minimum-necessary disclosure and documentation principles as other communication. Informal channels should not replace approved channels for sensitive information.

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6. Appointment Communication

Appointment messages should confirm date, time, location and approved preparation or attendance requirements, together with the cancellation or rescheduling route. Administrative staff should not add unapproved clinical instructions.

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7. Billing Communication

Billing staff should use approved billing information and explain administrative details clearly. Disputed charges should be routed to Finance. Staff should not guarantee coverage, reimbursement or unauthorized financial adjustments.

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8. Clinical Boundaries

Administrative employees should not interpret diagnoses, reports, medication instructions or test results. Such requests should be routed to the relevant clinical team. Administrative staff may provide information that is clearly within their role.

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9. Complaints

Complaint conversations should focus on understanding the concern, recording facts and explaining the next step. Staff should not state that an allegation is proven before review. Escalation should be used when the matter exceeds the employee's authority.

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10. Privacy

Sensitive information should be disclosed only after the required verification and only to the extent necessary for the purpose. Privacy concerns should use the designated Legal & Compliance route.

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11. Accessibility

Approved interpreter and accessibility processes should be used when needed. Staff should avoid unnecessary jargon and confirm understanding for important administrative instructions.

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12. Documentation

Material handoffs, commitments and escalations should be documented through designated workflows. Managers may review verification compliance, communication-related complaints, escalation completeness and documentation quality.

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13. Difficult Interactions

When an interaction becomes unsafe or cannot be resolved within the employee's authority, staff should use the approved escalation process. Employees should remain factual and avoid unsupported promises.

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14. Scenarios and FAQs

A caller who cannot verify identity should receive only information permitted without verification. A family member requesting sensitive information should follow the authorized disclosure process. A patient asking for result interpretation should be routed to clinical staff. A billing dispute should be routed to Finance.

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15. Governance

The guideline owner should review the document periodically and after material changes to communication channels, verification requirements, organizational responsibilities or applicable workflows.

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